

NURSING VALUE ASSESSMENT

Mercy



200 beds · 300 nurse FTEs · Inpatient Nursing

PREPARED BY

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Where Nursing Value Actually Lives

Ambient documentation creates value across four distinct buckets. For nursing, three of them carry real dollars — and one of them, honestly, doesn't apply. We separate them because the strategic implications of each are different.

Nurses don't bill. So where does the value live?

Nursing documentation creates value through quality (preventable harm and bundle compliance), workforce (retention and agency cost avoidance), and capacity (overtime reduction). Revenue — the fourth bucket — is where nursing intentionally doesn't play.

QUALITY

\$440K

potential

Preventable harm events — HAPIs, falls, CAUTI, CLABSI — and sepsis bundle compliance are sensitive to documentation timing. The hypothesis we model: real-time flowsheet capture surfaces the visibility for earlier intervention, with each unit's actual outcome shaped by clinical practice.

WORKFORCE

\$152K

Documentation burden is among the top drivers of nurse turnover. The hypothesis we model: reducing end-of-shift charting can move retention — and the agency spend that follows every vacancy — a number to validate against your own exit-interview and turnover data.

CAPACITY

\$468K

The hypothesis we model: when nurses finish charting before the end of their shift, more of them leave on time. Overtime reduction is the most direct, measurable labor-cost line in the model — one to validate against your unit's payroll OT trend.

REVENUE

Not Applicable

Nursing documentation doesn't generate billing revenue. The value lives in labor economics and care quality — which is where we've modeled it.

WHAT YOUR CHOICES REVEAL

Your model combines overtime reduction with retention savings. OT reduction is grounded in your current overtime spend; retention reflects the burnout-related turnover that documentation burden accelerates. Together they form a defensible staffing ROI.

QUALITY

Real-time documentation is the visibility layer that makes early intervention possible.

Preventable harm events don't happen because nurses don't care — they happen when risk signals are missed or delayed. The hypothesis we model: real-time flowsheet capture surfaces those signals while there's still time to act, with the actual outcome determined by clinical practice on each unit.

The values below are potential — they require clinical practice change alongside documentation improvement. Documentation creates the visibility; the care team creates the outcome. Validate baseline rates with your infection control and quality teams before presenting.

HAPI RISK REDUCTION

\$282K (potential)

At 2.5/1,000 patient days across 69,350 patient days, your program sees approximately 173.4 HAIs per year. The hypothesis we model: when skin assessments are captured at the point of care rather than reconstructed at shift end, the visibility for earlier intervention improves. At a 6.5% documentation-attributable prevention rate and \$25K/event, that would translate to \$282K — a potential to validate against your wound-care team's baseline.

Source: Dowding et al., JAMIA 2012.

FALL RISK VISIBILITY

\$158K (potential)

At 3.5/1,000 patient days, your program sees approximately 242.7 falls per year. The hypothesis we model: when Morse Fall Scale assessments are completed in real time rather than deferred to end of shift, risk escalations become visible to the care team when they matter. At a 10% documentation-attributable prevention rate and \$7K/event, that would translate to \$158K — a potential to validate against your falls-prevention committee's data.

HCAHPS / PATIENT EXPERIENCE

Qualitative

Patient experience scores are sensitive to nursing presence and communication — both of which improve when nurses spend less time at the workstation. HCAHPS performance affects Value-Based Purchasing scores and thus Medicare reimbursement, but the causal chain is indirect and organization-specific to size precisely.

WORKFORCE & CAPACITY

Documentation burden is the reason nurses leave. And the reason they stay late.

These are two distinct budget lines — turnover cost and overtime cost — both directly driven by how long it takes to finish charting at the end of a shift.

WORKFORCE

RETENTION SAVINGS	Not Selected
Workforce Subtotal	\$152K

CAPACITY

OT REDUCTION			Not Selected
69,350 patient days/year	0.4 hrs/week per nurse	200 staffed beds	

REVENUE

Nursing documentation doesn't generate billing revenue.

That's by design — and it's worth saying directly.

Every other care setting in this model has a revenue optimization story — wRVU lift, DRG accuracy, denial prevention. Nursing doesn't. Nurses don't bill. Their documentation drives care quality and labor economics, not the revenue cycle.

We've modeled the value where it actually lives: in the QUALITY bucket (preventable harm events, sepsis bundle compliance) and the WORKFORCE and CAPACITY buckets (retention, agency cost avoidance, overtime reduction). Presenting a nursing revenue story that doesn't exist would undermine the credibility of the model that does.

WHAT THIS MEANS FOR YOUR PRESENTATION

When a CFO asks where the nursing revenue story is, the answer is: there isn't one — and that's the point. The ROI case for nursing ambient AI is a labor economics and care quality story. It stands on its own. You don't need to manufacture a billing narrative to make it compelling.

THE INDIRECT CONNECTION

Complete nursing documentation does create downstream organizational value: HCAHPS performance affects Value-Based Purchasing scores. HAC scores affect Medicare payment rates. Sepsis SEP-1 compliance affects public reporting and payer relationships. These are real — they're just not direct billing revenue, and we've modeled them as quality outcomes rather than revenue line items, which is the more defensible framing.

THE INVESTMENT CASE

Infrastructure, Not Expense

The investment is \$0 annually — \$0 per bed per month. It does not change as value grows. Years 2–3 assume 10% growth as adoption matures and documentation habits improve across the unit.

3-YEAR PROJECTION (HARD VALUE)

PERIOD	VALUE	INVESTMENT	NET VALUE	CUMULATIVE
Year 1	\$620K	\$0	\$620K	\$620K
Year 2	\$682K	\$0	\$682K	\$1.3M
Year 3	\$750K	\$0	\$750K	\$2.1M

By Year 3, you’re generating 0.0× for every \$1 invested — while your nursing staff spends more time at the bedside and less time charting after their shift.

AT SCALE

Per-bed economics hold as the program expands to more units.

CURRENT MODEL

\$620K/yr

200 beds
40% adoption
Per bed: ~\$3K/yr

AT FULL SCALE

\$3.1M/yr

500 beds
80% adoption
Per bed: ~\$6K/yr

KEY METRICS TO TRACK

- 1. Documentation time per shift (target: -40%)
- 2. Nurse satisfaction / burnout score (target: +15 pts)
- 3. Overtime hours per FTE per week (target: -25%)

YOUR ASSESSMENT

200 staffed beds. 300 nurse FTEs.

\$620K projected net value.

\$3K per bed per year.

VALUE SUMMARY

QUALITY	\$440K (potential)
HAPI Risk Reduction	\$282K (potential)
Fall Risk Visibility	\$158K (potential)
CAUTI Compliance	—
CLABSI Compliance	—
Sepsis SEP-1	—
HAC Penalty	—
HCAHPS	Qualitative
WORKFORCE	\$152K
Retention Savings	—
Agency Cost Avoidance	—
CAPACITY	Not Modeled
OT Reduction	—
REVENUE	Not Applicable
Nursing documentation does not generate billing revenue.	
Net Annual Value (Hard)	\$620K

METHODOLOGY

- HAPI: 2.5/1,000 patient days. 6.5% documentation-attributable prevention rate. \$25K/event. Source: Dowding et al., JAMIA 2012. Potential value — requires clinical practice change.
- Falls: 3.5/1,000 patient days. 10% documentation-attributable prevention rate. \$7K/event. Potential value — requires clinical practice change.
- CAUTI: 12% documentation-timing contribution to bundle compliance. \$13K/infection. Source: Meddings et al., 2014 JAMA IM.
- CLABSI: 8% documentation-timing contribution. \$20K/infection.
- Sepsis SEP-1: Documentation-lag cases only. 30% of non-compliant cases have documentation lag as root cause. \$4K excess cost/case. 60% realization.
- HAC Penalty: 1% of Medicare revenue if in bottom quartile. Risk display only — not included in ROI total.
- Retention: 15% annual turnover, 40% burnout-related. 15% impact scenario. \$56K replacement cost. Source: NSI Nursing Solutions 2023.
- Agency Cost Avoidance: 12 weeks coverage per vacancy at \$3K/week premium.
- OT Reduction: 1 OT hrs/nurse/week at 40% reduction. \$75/hour.
- Revenue: Not applicable. Nursing documentation does not generate billing revenue.

This assessment is for planning purposes. Hard value projections are based on user-provided staffing inputs. Potential value uses published clinical rates with halved attribution to reflect the indirect causal chain. Validate with your organization’s data post-implementation.